



DEPARTMENT OF MEDICAL ONCOLOGY

Patient Name	: Mr. RAJEEV SAINI	UHID	: 300169346
Bed No	: 224-5-SS-M	IP. No.	: AC26/7825
Company	: SHA (U.P) - Credit	Age/Gender	: 41 Yrs 4 Mths 16 Days/Male
Contact No	: 7505860437	Admission Date	: 16/05/2026 04:40 PM
Unit Name	: Dr. JB Unit	Date of Discharge	: 17/05/2026
Doctor Name	: Dr. J B Sharma, Dr. Asmita Jain		
Address	: 1710 ANTA PARA MATHURA, MATHURA, Uttar Pradesh, INDIA, 281001		

DISCHARGE SUMMARY

HEIGHT :- 161CM WEIGHT:- 65KG BSA:- 1.70M2

1. DIAGNOSIS: DLBCL (GCB): HIGH GRADE B CELL LYMPHOMA APRIL 2026
STAGE: II, LDH (175), BONE MARROW BIOPSY: NORMAL
IPI SCORE 2 CNS RISK SCORE: 2, Ki 67:80%
CO MORBID CONDITIONS: NIL (LVEF-55%)
MO029B

CHEMOPLAN:- R-CHOP Q 3 WEEKLY X 3 CYCLES AND REASSESS

NJ. RITUXIMAB 375mg/M2 D1

NJ. CYCLOPHOSPHAMIDE 750mg/m2 D2

NJ. DOXORUBICIN 50mg/M2 D2

NJ. VCR 1.4mg/M2 IV D2

TAB. PREDNISOLONE 100mg PO X 5 DAYS Q 3 WEEKLY X 3 CYCLES AND REASSESS

2. KNOWN ALLERGIES: NO KNOWN DRUG ALLERGIES.

3. BRIEF SUMMARY OF CASE: Mr. Rajeev Saini 41years old male is a known case of DLBCL (GCB) high grade B cell lymphoma April 2026. He was investigated.

4. CT Whole body done on 02/04/2026 showed metabolically ative soft tissue density mass involving anterior nasopharynx-likely primary malignant pathology. Metabolically active multiple discrete and conglomerated cervical and irght axillary lymphnodes-likely metastatic. No other significant abnormal perimetabolic lesion in rest of the body surveyed.



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Biopsy from right axillary node done on 03/04/2026 showed nodal tissue cores. No evidence of granuloma or malignancy (IHC for CK is negative)

MRI Neck with contrast (05/04/2026) showed significant diffuse circumferential wall thickening of nasopharynx and oropharynx leading to the luminal narrowing. Multiple enlarged bilateral level II and left level III lymphnodes. Post contrast study reveals mild enhancement of thickening and lymphnodes.

IHC marker done (15/04/2026) features are suggestive of high grade B cell lymphoma likely Diffuse large B-cell lymphoma GCB type.

Bone Marrow Biopsy (23/04/2026) showed No morphological evidence of infiltration by lymphoma seen (in a K/C/O DLBCL).

2D Echo 24/04/2026 showed LVEF 55%.

He received C1 D1 of chemotherapy (Inj. Rituximab 600mg) (Multiple drug chemotherapy) on 25/04/2026 and C1 D2 of chemotherapy (Inj. Endoxan 1200mg + Inj. Doxorubicin 80mg + Inj. Biocristine 2mg) (Multiple drug chemotherapy) on 26/04/2026

Inj. Pegstim 6mg, which is given on 28/04/2026.

Presently he is admitted chemotherapy. He received C2 D1 of chemotherapy (Inj. Rituximab 600mg) (Multiple drug chemotherapy) on 16/05/2026 and C1 D2 of chemotherapy (Inj. Endoxan 1200mg + Inj. Doxorubicin 80mg + Inj. Biocristine 2mg) (Multiple drug chemotherapy) on 17/05/2026. Patient tolerated the treatment well



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and is now being discharged in stable condition.

5. PERSONAL HISTORY: Nothing significant

6. PAST HISTORY: No H/o HTN/DM/CAD/COPD

7. FAMILY HISTORY: Nothing significant

8. GENERAL EXAMINATION: Patient Conscious, Oriented, Afebrile, BP-130/80mmHg, PR-106/min, RR-20/min., Chest-B/L AE+, CVS-S1S2+, P/A-Soft, BS+.

9. INVESTIGATIONS: Lab reports attached

10. COURSE IN THE HOSPITAL:

MEDICINE GIVEN:- He received C2 D1 of chemotherapy (Inj. Rituximab 600mg) (Multiple drug chemotherapy) on 16/05/2026 and C1 D2 of chemotherapy (Inj. Endoxan 1200mg + Inj. Doxorubicin 80mg + Inj. Biocristine 2mg) (Multiple drug chemotherapy) on 17/05/2026 along with other supportive care.

11. CONDITION AT DISCHARGE: satisfactory

12. ADVICE ON DISCHARGE:

TAB. PREDNISOLONE 100mg ONCE DAILY FOR 5 DAYS

TAB. OMNACORTIL 50mg TWICE DAILY FOR 5 DAYS



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TAB. ACIVIR 400mg TWICE DAILY FOR 21 DAYS
TAB. ONDY MD 4MG SOS IF VOMITING
CAP. SONIRAB LS ONCE DAILY FOR 5 DAYS
CAP. VITAGRACE ONCE DAILY FOR 21 DAYS
CAP. COZIM Q TWICE DAILY FOR 21 DAYS
TAB. CALEAT KD ONCE DAILY FOR 21 DAYS
CAP. IROXY 150MG ONCE DAILY FOR 21 DAYS
CAP. PROBIART WICE DAILY FOR 21 DAYS
TAB. GOLDCAL D3 ONCE A WEEK FOR 4 WEEK
CAP. STREMZA TWICE DAILY FOR 21 DAYS
CAP. PHYCOVEL TWICE DAILY FOR 21 DAYS
SYP. CELQLIEF 5ML THRICE DAILY WITH WARM WATER FOR 21 DAYS
TAB. RALGYZ PLUS 250mg TWICE DAILY FOR 21 DAYS
PLENTY OF FLUIDS

TO CONTINUE MEDICATION FOR HIGH BLOODPRESSURE, DIABETES, HYPOTHYROIDISM
AND OTHER ACCOMPANYING DISEASE ADVISED BY YOUR CARDIOLOGIST/PHYSICIAN IF
APPLICABLE.

IN CASE OF LOOSE MOTION

TAB. OFLOX OZ TWICE DAILY
TAB. RIDOTIL THRICE DAILY
ELECTROL SACHET TWICE DAILY

H-2/FC-34, A-4, Paschim Vihar, New Delhi-110063

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Prepared By: Mr. Abdul Azad Ali

Finalize By: Mr. Abdul Azad Ali

A UNIT OF MANAV SEVARTH TRUST



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ECONORM SACHET THRICE DAILY
CAP. IMMIDIUM 2 CAPS STAT

IN CASE OF FEVER (TEMP. >100°F)

TAB. AUGMENTIN 625mg TWICE DAILY FOR 5 DAYS
TAB. DOLO 650mg THRICE DAILY

IN CASE OF VOMITING

TAB. EMESET 8mg THRICE DAILY
TAB. PERINORM THRICE DAILY

IN CASE OF CONSTIPATION

SYP. LOOZ 30ml AT BED TIME
TAB. DULCOLAX 2TABS STAT

DIET ADVISED:- Normal diet
plenty of oral fluids

Precautions:- As advised.

13. FOLLOWUP:- 1. REVIEW WITH DR. J.B. SHARMA ON 23/05/2026 WITH CBC REPORT AND ON 06/06/2026 WITH CBC, KFT, CALCIUM REPORTS FOR C3 D1/D2 CHEMOTHERAPY IN MEDICAL ONCOLOGY OPD.



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2. FOR OPD APPOINTMENT CALL ON 011-49222222.

IN CASE OF THE FOLLOWING MEDICAL PROBLEMS CONTACT YOUR DOCTOR OR VISIT CASUALTY

Persistent fever
Recurrent Vomiting
Bleeding from any site
Severe Abdominal pain etc

IN CASE OF EMERGENCY TO VISIT CASUALTY

DR. J. B. SHARMA
MD, DM(Medical Oncology)
DIRECTOR


DR. ASMITA JAIN
MD, DrNB(MEDICAL ONCOLOGY)
M.B.A.(HEALTHCARE), ECMO
CONSULTANT

The above treatment had been explained to me along with medications

Name of Patient/Attendant:

Signature of Patient/Attendant:

Relation with Patient:-